

Q I'm 26 weeks pregnant and my doctor is worried about the size of my baby. What will happen?

When a baby's growth slows or ceases in the uterus, this is known as intrauterine growth restriction (IUGR), which can be the result of a variety of factors. If your doctor is concerned about your baby's growth, he or she will arrange extra ultrasounds to see what the problem is and how it can be treated.

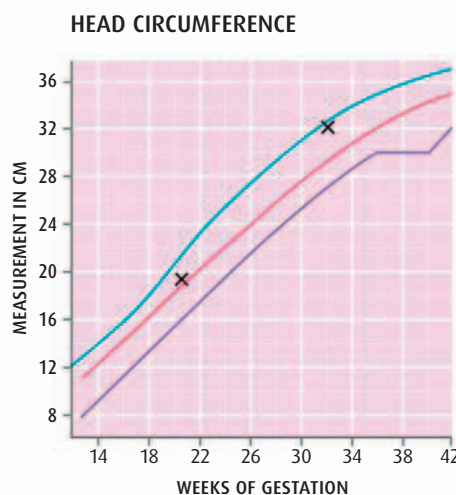
The size of a developing baby is measured according to percentile graphs (you'll have these to keep track of your baby's length and weight once she is born, too). From 24 weeks onward, these graphs can plot the average growth rate of a baby according to the height of the mother's fundus. This is the measurement your doctor makes during your prenatal appointments using a tape measure and is taken from the highest point of your belly under your sternum, over the top of the belly to your pubic bone.

If this measurement falls beneath the tenth percentile when you are between 26 and 28 weeks pregnant, your doctor will probably send you for additional ultrasounds so that your baby's skull diameter and abdominal circumference can be checked directly over a given amount of time (and these measurements plotted on relevant percentile graphs). The height of your fundus is influenced not only by your baby's

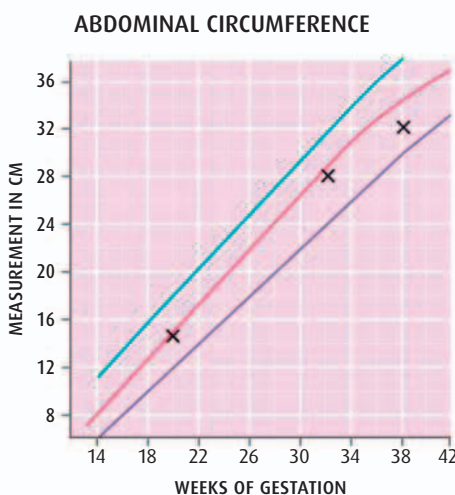


An ultrasound If your baby's growth appears to be slowing down, you may be offered more than one to assess progress more accurately.

size, but also by the amount of amniotic fluid you have—too little or too much fluid can also, rarely, be cause for concern. Having these growth ultrasounds is a precaution intended to reassure you and your doctor that all is well.



Growth graphs The graph on the left shows that a baby's head is growing steadily. The graph on the right indicates that the growth of the abdomen is showing decline possibly due to blood and nutrients being pumped to the heart and brain.



KEY

— 90th percentile	✕ Test results
— 50th percentile	
— 10th percentile	

Q What kind of situations mean that I will need to have additional ultrasounds and checks?

Some pregnancies require extra monitoring to ensure that the baby is growing well.

Sometimes this is because of issues identified at the beginning of pregnancy, and extra ultrasounds and checkups can be planned. Other problems arise during the course of pregnancy. The following conditions mean that extra checkups may be planned at your first appointment:

- » Complications in a previous pregnancy, such as a small baby or a stillbirth.
- » A preexisting medical condition, such as diabetes or a heart condition.

Situations that develop during pregnancy that may require additional monitoring include:

- » Being pregnant with multiples, which you are most likely to find out about at your first ultrasound.
- » Developing gestational diabetes.
- » If you are rhesus negative and you have developed anti-D antibodies.

The following situations mean that you may need extra ultrasound(s) toward the end of pregnancy to check your baby's well-being:

- » If your water breaks before your due date.
- » Having too much or too little amniotic fluid.
- » Being two weeks past your due date.
- » If your baby stops her normal movements.

Q My doctor mentioned a Doppler ultrasound. Why would I need this?

If you have complications in your pregnancy, or an ultrasound confirms that there is a problem with your baby's growth, you may be given a Doppler ultrasound. This scan measures blood flow in the umbilical artery and uterus and determines the placental health. You may have more than one Doppler ultrasound during your pregnancy to monitor your baby's growth over a period of time. If there are concerns about your baby's movements, a type of Doppler ultrasound known as a cardiotocograph may be done that traces your baby's heartbeat, and depending on the results, an early induction of labor may be advised. Cardiotocographs are typically done in the third trimester, but can be done earlier.